Operational Standards

Operational standards are based on best practices and guidelines to ensure the safety, accuracy, and efficiency of the phlebotomy process. These operational standards help keep patients and phlebotomists safe, ensure the collected samples are collected correctly, and provide accurate results. Agencies or organizations such as The Joint Commission (TJC), the Clinical and Laboratory Standards Institutes (CLSI), and the CDC create and enforce these standards. The Joint Commission's primary mission is to ensure safety in health care. This is done by establishing standards, promoting safety, and encouraging continued education. TJC puts together National Patient Safety Goals each year to help ensure patient safety at all times. Some of their goals include improving the accuracy of patient identification and enhancing health care equity. The CLSI assists with developing standards around the world to improve medical laboratory testing and patient care. CLSI helps develop best practices and promotes standardization. The CDC develops and enforces standards around infection control, emergency preparedness, and prevention of disease transmission. Some of the CDC's standards focus on proper hand washing, safety of sharps, and universal precautions.

Some common operational standards include proper patient identification and having the correct supplies or equipment. Proper technique is essential for all aseptic and venipuncture procedures and specimen handling. A patient must be identified immediately before any procedure occurs. Following the guidelines, the patient must be identified by at least two patient identifiers. This could include the patient's first and last name, and date of birth, which can be compared to the requisition or the printed wrist or ankle band. In an inpatient hospital setting, a barcode on the patient's wristband may be scanned to identify them. Double-checking the correct supplies before beginning the drawing process can help reduce errors and the need to redraw a patient. Confirm that all supplies are not expired or unopened, and that the correct ETS tubes are chosen to match the tests ordered. If an incorrect tube color is collected, it may not be able to be used due to the impact of the preservative on the blood in the tube. The patient's sample is then required to be recollected. Another operational standard, correct technique, can reduce harm to the patient. Selecting the correct site and using the correct collection technique allows for collecting high-quality specimens. Proper hand hygiene and ***aseptic technique*** help maintain as much sterility as possible, reducing the risk of infection. Following proper specimen handling guidelines helps ensure the sample remains as close to the patient's condition as possible. Ensuring that tubes are labeled immediately and correctly before leaving the patient helps reduce the risk of misidentification of the sample. Have the patient check that their name, date of birth, or other identifying information on the label is correct. After the tubes are labeled, ensure they are stored appropriately until they are brought or transported to the lab. Always check if a sample needs to be room temperature, refrigerated, or frozen and follow all manufacturer's guidelines.

aseptic technique. A practice used to reduce the chance of microbial contamination.